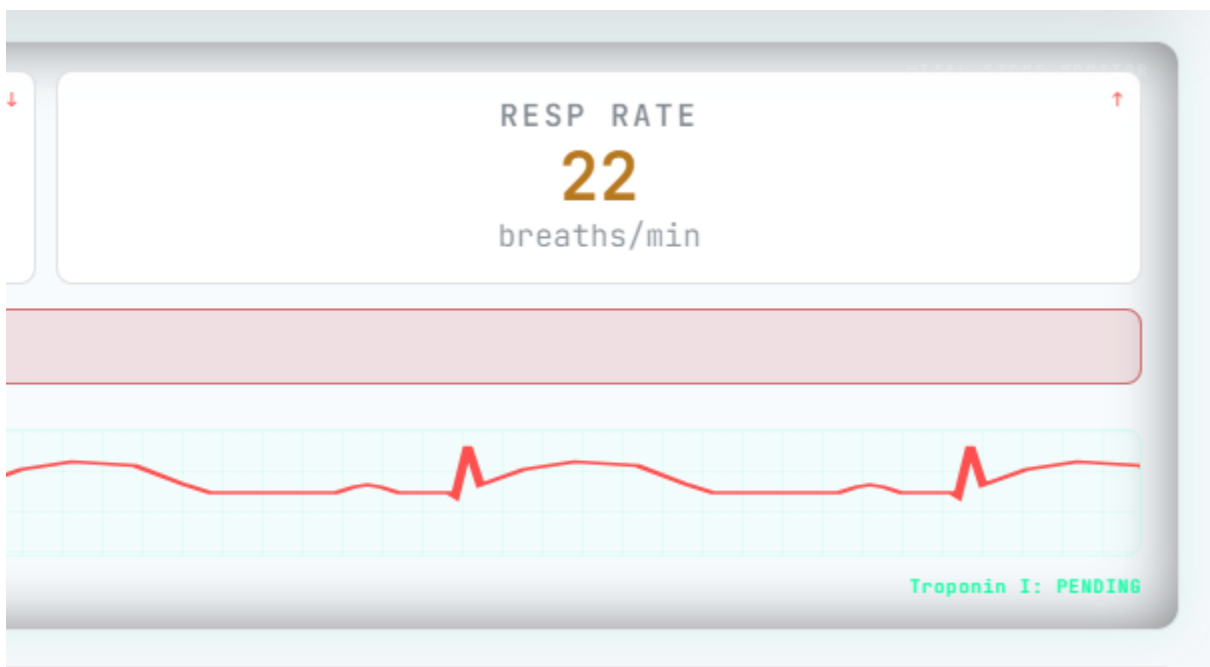


I viewed it on a monitor screen on a PC, so the thoughts are based on that. Ideally the patient's situation when he walked in should be more clearly readable. All the other metrics are more or less clear but overall the read is a problem.

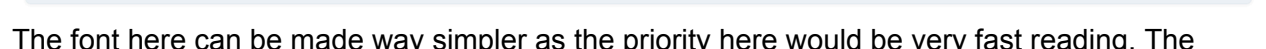


(Troponin:Pending)- Not sure whether this is crucial information, but I would assume that everything should be readable.

Overall there is a lack of hierarchy to the reading of information, a little overwhelming to look at, maybe also because I am not aware of what these metrics actually mean, but there is a certain clutter.



and if we are keeping it the size difference in the



COGNITA: Gap flagged. Priority coaching activated.

This information is not required for the user, again adding to clutter.

TEACHING POINT - UNDERSTANDING THE EXPERT RESPONSE

21% 21% of nurses made the same choice here

DT: SBAR

Dr. Adewale

Incomplete SBAR – the Recommendation (“I need a verbal order for cath lab activation now”) is the part that gets the order. Without it you get questions back.

EVIDENCE

Kaiser SBAR research: complete SBAR reduces physician callback time by 40% versus unstructured handoffs.

CONTINUE

The type hierarchy needs to be taken care of so that the information is very visible and readable easily. Right now they are all of similar sizes and very easy to get lost. Since this is the most crucial aspect of the loop, I feel it's very important to fix typography here.

I felt mostly there is a readability issues at many points due to the size of text and a little bit of hierarchy can make the whole thing way more user friendly.

QUESTION

Dr. Martinez (ED attending) is occupied with a trauma code. Mr. Osei is hypotensive, BP dropping. What are your **first three actions**?

ANSWER

A

Give aspirin 325mg, apply O₂ at 4L, page attending start for STEMI review. Do not activate cath lab immediately — await physician confirmation before any irreversible escalation.

B

Recheck vitals and reassess before acting.
He arrived ambulatory and talked through history — situation may be less urgent than the ECG pattern indicates. Repeat V4–V5 leads.

C

Give morphine IV for pain and administer SL nitroglycerin for chest pressure
Hold aspirin until allergy history confirmed — ECG changes could represent demand ischemia.

D

Give aspirin 325mg, activate cath lab per ECG criteria — no physician required. Hold supplemental O₂ (SpO₂ >94%) — indicated only if <90%. Page attending simultaneously. NTG is contraindicated at SBP <92.

CRITICAL GAP:
Morphine worsens ACS outcomes; NTG contraindicated at SBP <92

ANSWER EXPLANATION

1. Recheck vitals and reassess before acting. The patient is hypotensive (SBP 90 mmHg) and has a concerning ECG (V4-V5 leads). Before initiating any treatment, it is crucial to reassess the patient's vital signs and symptoms. The patient's hypotension suggests a potential complication, and the ECG findings are not definitive for STEMI. Administering aspirin or nitroglycerin without further assessment could be harmful, especially in the context of hypotension.

2. Give aspirin 325mg, apply O₂ at 4L, page attending start for STEMI review. Aspirin is a standard initial treatment for suspected ACS. Applying O₂ at 4L/min is appropriate for patients with hypoxia. Pageing the attending physician is essential for further evaluation and management.

3. Do not activate cath lab immediately — await physician confirmation before any irreversible escalation. Catheterization is a high-risk procedure. Given the patient's hypotension and the non-definitive ECG, it is prudent to wait for the attending physician's confirmation before proceeding with catheterization.

CRITICAL GAP

Morphine worsens ACS outcomes; NTG contraindicated at SBP <92

I'll share this image also separately since it's not very clear here to illustrate my point. I didn't know what fonts you guys use, so I just used whatever I had that could fit here. Anyway, my point is, this second version is instantly more readable and easier to look at. The information is the same. Also the gap in learning is incorporated in the wrong answer itself which I think is less cluttered and also helps in clear understanding rather than using a major chunk of space on the page with an extra box. The box is also largely empty so a lot of space is also wasted while making it difficult to look at. Just did it on two options to show the difference. Similarly in a lot of areas on the page, we can use text hierarchy to make information a lot more clearer and more simple to look at.